TSHR and IGF1R in Graves disease and thyroid eye disease: genetic evidence from blood expression

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Abstract

Objective: Thyroid eye disease (TED) can accompany Graves disease (GD), an autoimmune cause of hyperthyroidism. We compared genetic evidence linking blood *TSHR* and *IGF1R* expression with susceptibility.

Methods: We screened 2,544 druggable genes using Mendelian randomization (MR). Disease outcomes were Biobank Japan (BBJ) GD, UK Biobank (UKB) hyperthyroidism and overlapping FinnGen GD, Graves ophthalmopathy (GO) and strict GO endpoints. Colocalization tested 24 genes for a shared causal variant. Tissue eQTL and orbital RNA-sequencing (four TED, one control) were supplementary.

Results: Thirteen genes passed BBJ discovery ($P < 0.05/2{,}544$). Higher genetically proxied *TSHR* expression was associated with lower GD odds in BBJ (odds ratio [OR] per approximately one standard deviation 0.12, 95% confidence interval [CI] 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$), concordantly across all five datasets. Shared-variant probability exceeded 0.80 in FinnGen GO and, prior-sensitively, BBJ and strict GO, but UKB (0.226) and FinnGen GD (0.264) favored distinct variants. *IGF1R* ORs exceeded 1 but reached $P < 0.05$ only in BBJ (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB ($P = 0.0117$); the most precise estimate, FinnGen GD, was OR 1.07 (95% CI 0.86–1.33). Single-variant tissue estimates added no independent evidence. Mean orbital *TSHR* and *IGF1R* transcript levels exceeded the control.

Conclusions: Association evidence was stronger for *TSHR* than *IGF1R*, but absent colocalization in UKB and FinnGen GD leaves expression-mediated causality unproven. Population-controlled data cannot isolate TED susceptibility within GD; weaker *IGF1R* evidence does not argue against IGF-1R–targeted treatment.

Keywords: Graves disease; thyroid eye disease; *TSHR*; *IGF1R*; Mendelian randomization; colocalization

Introduction

Thyroid eye disease (TED) can cause proptosis, diplopia and visual impairment in patients with Graves disease (GD) [1, 2]. Teprotumumab, which blocks the insulin-like growth factor 1 receptor (IGF-1R), improves clinical outcomes in active TED [3]. Its efficacy raises a related but distinct question: does inherited variation influencing *IGF1R* expression contribute to disease susceptibility in the same way as variation at *TSHR*, the established GD autoantigen and susceptibility locus?

TSHR and IGF-1R interact in experimental models of orbital disease [4, 5, 6]. However, treatment response, inherited gene regulation and gene expression in inflamed tissue address different biological questions. A receptor may be a useful treatment target without equally strong genetic evidence linking its expression to disease onset. Distinguishing these forms of evidence helps clinicians interpret genetic studies of therapeutic targets.

Mendelian randomization (MR) uses variants associated with an exposure to investigate its relationship with the disease phenotype being studied, termed the outcome [7]. Here, the exposure is blood gene expression and the outcomes are disease diagnoses, not responses to treatment. Because alleles are allocated at conception, MR is less prone than ordinary observational studies to confounding and reverse causation, but it supports causal interpretation only when its instrumental-variable assumptions hold. Colocalization asks whether expression and disease associations in a genomic region are consistent with the same causal variant [8]. Distinct nearby variants can appear connected because they are inherited together. Thus, an MR association may be highly significant even when the regional signals do not support a shared variant. MR reports odds ratios (ORs), confidence intervals (CIs) and *P* values; colocalization reports model-based posterior probabilities, which are not *P* values.

Previous genetic studies have already implicated *TSHR* in GD and Graves ophthalmopathy. Ji and colleagues integrated blood methylation, expression and protein associations with GD, reporting *TSHR* among the expression-associated genes also supported in a FinnGen ophthalmopathy dataset [9]. Other MR work has examined inflammatory proteins in ophthalmopathy [10], and recent integrative research has investigated *TSHR* across genetic and transcriptomic data in GD and ophthalmopathy [11]. These findings establish a basis for comparing clinically relevant receptors, but do not determine whether evidence for their blood expression is consistently supported across disease definitions under the same analysis framework. In particular, limited evidence for *IGF1R* could reflect imprecision or dependence on particular instruments, while a strong MR association for *TSHR* does not by itself establish that expression and disease share a causal variant. The remaining question is how robustly these two expression traits are associated with GD and related phenotypes, and how far those associations support biological interpretation.

To address this question, we compared genetically proxied blood expression of *TSHR* and *IGF1R* — expression predicted from inherited variants rather than measured directly — within a screen of genes encoding potentially druggable proteins, using *CTLA4* and other associated genes as context. We used Biobank Japan (BBJ) GD for discovery, compared the original findings with UK Biobank (UKB) hyperthyroidism and FinnGen ophthalmopathy, and subsequently added FinnGen GD and a stricter ophthalmopathy definition. We evaluated effect direction, precision and statistical support across these datasets, then examined dependence on individual instruments, allele frequencies and colocalization assumptions. A later fixed-panel tissue extension evaluated whether these patterns extended beyond the original blood instruments. Our aim was to characterize the consistency and limits of expression-related genetic evidence for the two receptors. The analyses distinguish GD from broader hyperthyroidism and population-controlled ophthalmopathy; they do not directly

estimate TED susceptibility among patients who already have GD or predict treatment response. A small in-house orbital RNA-sequencing series (four TED samples and one control) is reported only as descriptive context.

Methods

Study design and disease datasets

This two-sample MR study used public association summary statistics and followed STROBE-MR reporting guidance [12]. The exposure used whole-blood expression quantitative trait locus (eQTL) statistics from eQTLGen, comprising up to 31,684 participants, predominantly of European ancestry [13]. The original outcome hierarchy comprised BBJ GD, UKB self-reported hyperthyroidism or thyrotoxicosis, and FinnGen R12 Graves ophthalmopathy [14, 15, 16]. We subsequently added FinnGen R12 E4_GRAVES_STRICT and E4_GRAVES_OPHT_STRICT. The five datasets and their case and control counts are shown in Table 1. The UKB source did not establish an exclusively European analysis sample, so ancestry was not inferred from the reference panel used in its source study.

BBJ remained the discovery dataset. FinnGen GD provided a same-disease comparison, UKB a broader-phenotype comparison, and the two ophthalmopathy endpoints sensitivity analyses of a TED-enriched phenotype. The three FinnGen endpoints contain overlapping participants and were not treated as independent replications or combined by meta-analysis. Their controls were population non-cases for the respective endpoint, not GD patients without TED. The FinnGen extensions, expanded candidate assessment and instrument diagnostics were post hoc; no prospective registration was available.

Expression instruments and MR

From 4,462 potentially druggable genes [17], instruments were selected from significant blood cis-eQTLs ($P < 5\times10^{-8}$) within 1 Mb of the gene. PLINK clumping used a linkage disequilibrium (LD) threshold of $r^2 < 0.001$ within 10 Mb and 503 European participants from 1000 Genomes Phase 3 [18, 19]. This produced 6,135 single-nucleotide polymorphism (SNP)–gene records for 2,544 genes. The minimum F statistic was 29.7; the median variant-specific expression sample size was 28,092, with a range of 232–31,684. Exposure coefficients and standard errors were reconstructed from Z statistics, sample sizes and reference allele frequencies under unit expression variance [20]. Alleles were aligned with TwoSampleMR; ambiguous palindromic or unavailable variants were excluded. UKB linear-model coefficients and standard errors were divided by $\mu(1-\mu)$, where $\mu = 3{,}731/484{,}598$, to approximate a log-odds scale. BBJ and FinnGen coefficients already used log odds.

We used Wald ratios for single-instrument genes and multiplicative random-effects inverse-variance weighting (IVW), with correction for under-dispersion, for multiple instruments [21]. An instrumental variable (IV) is a genetic variant used to represent variation in expression. Results are reported as ORs per approximately one standard deviation higher blood expression, with 95% CIs and two-sided *P* values. This scale describes reconstructed expression effects, not a drug dose or an intervention. Primary discovery required $P < 0.05/2{,}544$ (1.965×10^{-5}), retaining all eligible genes in the denominator even when an outcome estimate was unavailable. The 13 BBJ discoveries were assessed in FinnGen GD using a focused threshold of 0.05/13 and concordant effect direction. A separate full FinnGen screen used 0.05/2,544 and was exploratory. Nominal $P < 0.05$ was reported descriptively and was not equated with discovery significance.

MR interpretation assumes that instruments predict expression, are not associated with exposure–disease confounders, and influence disease only through the modeled exposure. A small *P* value or high PP.H4 cannot by itself establish these assumptions.

Where applicable, sensitivity assessment included Cochran's Q, alternative MR estimators and omission of individual instruments [22, 23]. Previously completed alternative-estimator results (MR-Egger, weighted median and weighted mode) for the original three outcomes were retained (Supplementary Dataset, MR_estimators_original); they were not represented as new analyses of every endpoint. Post hoc leave-one-out results assessed dependence on individual variants, not independent replication. Single-instrument *TSHR* results could not undergo these omission or multi-instrument pleiotropy tests.

In a post hoc allele-direction audit, we re-extracted rs179252 and rs179247 from the original expression and disease files. Disease effects were aligned to the blood expression-increasing allele, and G-allele correlations were calculated in European and East Asian reference genotypes and independently checked with PLINK. This audit assessed allele alignment and consistency with published thymus findings, rather than thymic mediation (Supplementary Dataset, TSHR_allele_audit).

Colocalization and robustness assessment

We report MR associations as the main findings and use colocalization to assess their interpretation. In the original analysis, colocalization was reported alongside MR as a parallel evidence layer; the present emphasis was adopted during post hoc reappraisal and was not a prospectively registered hierarchy. Colocalization used complete available regional cis-eQTL statistics, including non-significant variants, matched to each disease dataset [8]. The expanded set included all 13 BBJ discoveries, *IGF1R*, nine additional FinnGen GD discoveries and *HLA-G*, which was selected because its MR significance changed after

frequency substitution: 24 genes in total. We used coloc.abf with disease as dataset 1 and expression as dataset 2. Posterior probability of hypothesis 4 (PP.H4) measures support for a shared causal variant under the fitted model. PP.H3 instead supports distinct variants. We used PP.H4 ≥ 0.80 as a descriptive strong-support threshold, with priors $p1 = p2 = 10^{-4}$ and $p12 = 10^{-5}$, and repeated analyses at $p12 = 5\times10^{-6}$ and 10^{-6} [24]. All five hypotheses are defined in Supplementary Methods. Low PP.H4 alone does not establish no disease association, and H3 does not prove multiple independent signals.

We evaluated reference and eQTLGen cohort frequencies, distinguishing changes in effect scaling from changes in variant availability and harmonization. The 24-gene colocalization dataset contains 1,440 settings: 24 genes × five outcomes × four frequency/availability scenarios × three priors. For *TSHR*, we additionally examined narrower windows, a common five-outcome variant set, raw regional disease peaks and all available source cis-eQTLs. For newly evaluated candidates, we checked marginal signal coverage, shared instruments and variant aliases. *C4B* and *HLA-G* underwent instrument-level frequency, weighting, heterogeneity and omission checks. Full-precision results are supplied in one Supplementary Dataset workbook. The original BBJ–ophthalmopathy combined filter is retained as a historical descriptive comparison, not promoted to a prospectively specified validation rule or the primary conclusion of this reappraisal.

Tissue-specific extension

In a post hoc extension, we assessed both genes in a fixed panel of five GTEx v8 tissues: thyroid, cultured fibroblasts, subcutaneous adipose, visceral adipose and whole blood [25]. The extension plan was fixed before retrieving these tissue results, after the blood analysis was known. GTEx participants were predominantly of European ancestry, not exclusively European. Instruments required cis-eQTL $P < 5\times10^{-8}$, $F > 10$ and European-reference LD $r^2 < 0.001$. Ten planned gene–tissue comparisons against FinnGen GD formed the primary

family ($P < 0.005$); 40 comparisons against the other four datasets formed a secondary family ($P < 0.00125$). Unestimable combinations remained in the correction denominators. Effects used the original slopes and standard errors for inverse-normal-transformed expression. Complete regional summaries were not obtained for this extension, so tissue colocalization was not performed; significant-only pairs were used solely for instrument selection. Details and all planned combinations are in Table S3 and the Supplementary Dataset.

Orbital expression and implementation

An in-house orbital RNA-sequencing series of four patients with inactive TED and one control provided descriptive context only (Supplementary Methods). Technical replicates were combined by biological sample, and transcript abundance was displayed without differential-expression significance testing (Figure S1). These observations were not used to select genes or validate the genetic findings. Analyses used R 4.3.3, TwoSampleMR 0.7.4, coloc 5.2.3 and PLINK 1.9, with independent numerical verification in Python. No age- or sex-stratified patient analyses, individual-level multivariable regression, Steiger directionality test or multi-signal colocalization was performed.

Results

Discovery across druggable genes

Of 2,544 instrumented genes, 2,234 were estimable in BBJ, 2,505 in UKB and 2,480 in each FinnGen endpoint. Unavailable estimates were not counted as negative findings. Thirteen genes met the BBJ discovery threshold (Figure 1; Table 3A): *HLA-A*, *HLA-DQA2*, *CTLA4*, *TSHR*, *C4A*, *HSD3B7*, *TUBB*, *VKORC1*, *TNFSF14*, *PRSS36*, *MAPKAPK5*, *PSMB8* and *IFNGR1*.

TSHR and IGF1R across disease datasets

Higher genetically proxied blood *TSHR* expression was associated with lower odds of BBJ GD (OR 0.12, 95% CI 0.07–0.21; $P = 1.09\times10^{-14}$) and FinnGen GD (OR 0.08, 95% CI 0.05–0.12; $P = 3.19\times10^{-32}$). The same direction was observed for UKB hyperthyroidism (OR 0.09, 95% CI 0.06–0.14; $P = 8.77\times10^{-28}$), FinnGen ophthalmopathy (OR 0.10, 95% CI 0.04–0.24; $P = 2.82\times10^{-7}$) and strict ophthalmopathy (OR 0.15, 95% CI 0.06–0.39; $P = 9.02\times10^{-5}$). All five estimates used rs179252 as the single instrument (Table 2; Figure 2). The strict ophthalmopathy association reached $P < 0.05$ but not the full-screen threshold of 1.965×10^{-5}.

The expression-increasing rs179252-G allele was associated with lower odds in all five disease datasets and was positively correlated with rs179247-G in European (r = 0.978; r^2 = 0.957; n = 503) and East Asian (r = 0.996; r^2 = 0.992; n = 504) reference samples. The latter allele also increased blood *TSHR* expression. Allele-aligned recalculation reproduced the original MR estimates (Supplementary Dataset, TSHR_allele_audit).

All five *IGF1R* ORs exceeded 1, but no association met the full-screen threshold. BBJ GD (OR 1.56, 95% CI 1.07–2.28; $P = 0.0212$) and UKB hyperthyroidism (OR 1.35, 95% CI 1.07–1.70; $P = 0.0117$) reached nominal significance, meaning uncorrected $P < 0.05$. The FinnGen estimates did not: GD OR 1.07 (95% CI 0.86–1.33; $P = 0.539$), ophthalmopathy OR 1.41 (95% CI 0.85–2.33; $P = 0.182$) and strict ophthalmopathy OR 1.52 (95% CI 0.92–2.49; $P = 0.0996$). The FinnGen GD estimate was the most precise and lay close to the null. Weighted-median estimates resembled the IVW results in BBJ (OR 1.56; $P = 0.0288$) and UKB (OR 1.42; $P = 0.00244$), whereas MR-Egger and weighted-mode estimates were imprecise ($P \geq 0.32$), and MR-Egger intercepts were non-significant (P = 0.66, 0.56 and 0.43 in BBJ, UKB and FinnGen ophthalmopathy), although these tests have little power with three or four instruments. These findings show directional consistency without statistically convincing confirmation in FinnGen. They do not establish absence of an association.

Comparison with other associated genes

Four genes passed the full-screen threshold in both GD datasets: *TSHR*, *CTLA4*, *HLA-DQA2* and *TUBB* (Table 3A). In FinnGen GD, *CTLA4* was associated with lower odds (OR 0.17, 95% CI 0.08–0.36; $P = 2.15\times10^{-6}$), whereas *HLA-DQA2* (OR 1.90, 95% CI 1.64–2.20; $P = 1.41\times10^{-17}$) and *TUBB* (OR 3.28, 95% CI 2.09–5.17; $P = 2.68\times10^{-7}$) were associated with higher odds. *HLA-DQA2* and *TUBB* lie in the major histocompatibility complex (MHC) region. *CTLA4*, an established autoimmune locus [26], and *TUBB* had direction-consistent nominal associations across all five datasets. *HLA-DQA2* also showed higher-odds associations in both ophthalmopathy endpoints, but the UKB estimate was non-significant and in the opposite direction.

Using the focused FinnGen GD threshold of $P < 0.05/13$, seven of the 13 BBJ discoveries had direction-consistent associations. The three additional genes were *C4A* ($P = 4.97\times10^{-5}$), *TNFSF14* ($P = 0.00361$) and *IFNGR1* ($P = 0.00330$). *VKORC1* was not estimable in FinnGen GD; the denominator remained 13. Although *HLA-A* had the smallest BBJ *P* value (2.60×10^{-23}), its FinnGen GD association was non-significant ($P = 0.648$). Thus, discovery significance alone did not determine consistency across datasets.

The full FinnGen GD screen identified 13 significant genes, nine outside the BBJ discovery set: *APOM*, *NEU1*, *IGLV2-18*, *IGLV3-12*, *TNF*, *HIST1H4E*, *LTA*, *LTB* and *C4B* (Table 3B). Several shared the same instrument: *APOM* and *NEU1* used rs3101018, and *IGLV2-18* and *IGLV3-12* used rs5759410. These pairs therefore do not represent independent genetic findings. Seven of the nine (*APOM*, *NEU1*, *TNF*, *HIST1H4E*, *LTA*, *LTB* and *C4B*) were instrumented within the extended MHC region on chromosome 6. Five MHC-region genes among the BBJ discoveries were also retained in follow-up; membership of this region was not an exclusion criterion.

Instrument and frequency sensitivity

The two-instrument *CTLA4* estimates were largely supported by rs13030124. Omitting it from FinnGen GD left an imprecise estimate ($P = 0.323$), consistent with the dependence previously observed in UKB and original FinnGen ophthalmopathy. Heterogeneity was present in FinnGen GD for *C4A* (Q-test $P = 1.27\times10^{-12}$), *TUBB* ($P = 3.67\times10^{-5}$), *CTLA4* ($P = 0.0275$) and *HLA-A* ($P = 0.0387$). For the first three, cross-dataset associations should therefore be distinguished from agreement among instruments. In BBJ and UKB, two of the four single-variant omissions removed nominal *IGF1R* significance: rs2654980 in both ($P =$ 0.0751 and 0.468), rs59467480 in BBJ ($P = 0.0503$) and rs117212126 in UKB ($P = 0.0529$); FinnGen estimates had not reached nominal significance. This indicates sensitivity to instrument removal, which can reflect changes in precision as well as the contribution of the omitted variant; it does not by itself identify an invalid instrument (Figure S2; Supplementary Dataset).

Frequency substitution preserved the original 13 BBJ discoveries and the principal *TSHR* and *IGF1R* interpretation (Supplementary Dataset, MR_original_freq and MR_new_freq), but not every exploratory FinnGen classification (Table 3C; Table S2). For *C4B* in FinnGen GD, the MR *P* value changed from 1.10×10^{-5} to 1.48×10^{-4}, crossing the full-screen threshold; both versions had Q-test $P < 10^{-6}$. For *HLA-G*, *P* changed from 0.107 to 1.72×10^{-5} as the IVW weight of rs150254595 increased from 21.3% to 58.9%. Omitting this variant left non-significant estimates under either frequency source. Independent calculations reproduced the eight full-set and 72 instrument-omission estimates for these two genes. Precision also varied across the screen. At the discovery threshold, 14.6% and 35.6% of estimable BBJ genes had 80% power to detect ORs of 1.5 and 2.0 under a normal-approximation benchmark; the corresponding FinnGen GD proportions were 24.8% and 52.2% (BBJ: Supplementary Dataset, Detection_original; FinnGen GD: Supplementary Methods).

Colocalization findings

Regional analyses tested whether the MR associations reflected a variant shared with expression (Figures 2 and 3; Table S1). For *TSHR*, PP.H4 was 0.951 in BBJ, 0.986 in FinnGen ophthalmopathy and 0.891 in strict ophthalmopathy under the primary prior. It was lower in UKB (0.226) and FinnGen GD (0.264), where PP.H3 was 0.774 and 0.736, respectively. The latter patterns favored distinct variants under the fitted model despite very small MR *P* values. This discordance persisted in narrower regions and a common five-outcome variant set. The matched analyses retained the regional disease peaks, and all three FinnGen comparisons shared 6,579 variants in the full cis-region. Lowering the shared-variant prior p12 from 10^{-5} to 10^{-6} reduced PP.H4 to 0.661 in BBJ, 0.875 in ophthalmopathy and 0.451 in strict ophthalmopathy.

IGF1R PP.H4 ranged from 0.010 to 0.404. In UKB, expression-only, distinct-variant and shared-variant probabilities were 0.400, 0.196 and 0.404, respectively. *CTLA4* PP.H4 was 0.201 in BBJ and 0.254 in FinnGen GD, compared with 0.953 in UKB and 0.978 and 0.979 in the two ophthalmopathy endpoints. Neither *HLA-DQA2* nor *TUBB* reached PP.H4 of 0.001 in any dataset. No selected gene reached PP.H4 ≥ 0.80 in FinnGen GD; this statement applies to the 24 genes assessed, not all 2,544 screened genes. *TNFSF14* and *IFNGR1* had strong BBJ support (0.994 and 0.989) without consistent support across the other definitions. For the two *IGLV* genes in BBJ, the absent lead expression variant and weak overlapping expression signals limited interpretation of low PP.H4. The nine additional FinnGen discoveries and sensitivity-selected *HLA-G* lacked strong support in every dataset. The chromosome 16p11.2 discoveries remained compatible with a shared regional association rather than three independent expression targets.

Tissue-specific extension

Four of ten gene–tissue combinations provided a qualifying instrument, each retaining one SNP after clumping (F = 32.1–61.1). Higher genetically proxied *TSHR* expression in subcutaneous and visceral adipose was associated with lower FinnGen GD odds (OR 0.36, 95% CI 0.31–0.43; $P = 1.28\times10^{-34}$ and OR 0.50, 95% CI 0.43–0.58; $P = 4.09\times10^{-19}$, respectively), with corrected support in BBJ and UKB. Subcutaneous estimates also passed correction in both ophthalmopathy datasets; visceral estimates did not ($P = 0.0368$ and 0.186). The two *TSHR* instruments, rs12101255 and rs74064796, lie 50 bp apart in *TSHR* intron 1, about 15 kb from rs179252 (European r^2 0.46 for rs12101255), and both are themselves blood *TSHR* eQTLs (eQTLGen $P = 4.1\times10^{-34}$ and 6.5×10^{-34}). With one instrument, each tissue *P* value equals that variant's disease-association z-test *P* value, so these estimates re-express the regional *TSHR* association rather than providing independent evidence. Neither thyroid nor cultured-fibroblast *IGF1R* estimates reached $P < 0.05$ in any dataset. Their FinnGen GD estimates were OR 0.76 (95% CI 0.56–1.02; $P = 0.0692$), opposite in direction to the blood estimates, and OR 1.11 (95% CI 0.89–1.38; $P = 0.359$). All 20 estimates and 30 unestimable comparisons are reported (Table S3; Supplementary Dataset, Tissue_extension).

Descriptive orbital expression

Mean orbital *TSHR*, *IGF1R* and *CTLA4* transcript levels in four TED samples exceeded those in the single control (Figure S1). With one control and no confirmatory expression testing, these observations do not establish differential expression or explain the genetic associations.

Discussion

This study addresses how consistently blood expression-related genetic evidence supports two clinically relevant receptors across GD and related disease definitions. *TSHR* showed associations with lower disease odds in both GD datasets, with concordant directions in

broader hyperthyroidism and ophthalmopathy. All *IGF1R* ORs exceeded 1, but only the BBJ and UKB estimates reached uncorrected $P < 0.05$, none met the discovery threshold, and the most precise estimate lay close to the null. Colocalization supported a shared variant for *TSHR* in BBJ and both ophthalmopathy endpoints but favored distinct variants in UKB and FinnGen GD, the two datasets with the most cases. The contribution is a comparison of the reproducibility and limitations of these associations within the same analytical framework. It does not establish a difference in biological importance between the receptors, and statistical significance in one gene but not another is not a formal test of different effects.

Our findings extend an existing body of genetic evidence. Ji and colleagues had already linked blood *TSHR* expression to GD in an earlier FinnGen release and reported support in FinnGen ophthalmopathy [9]; recent integrative work has also investigated *TSHR* in GD and ophthalmopathy [11]. We add a focused comparison with *IGF1R*, a GD comparison spanning BBJ and FinnGen, and an assessment of both genes alongside the wider druggable-gene screen. The result is consistent association evidence for *TSHR*, limited evidence for *IGF1R*, and an explicit distinction between strong MR associations and variable regional support for expression-mediated causality. This comparison uses shared public resources, including eQTLGen and FinnGen, so agreement with earlier work is not wholly independent replication. Different MR methods, FinnGen releases and colocalization settings also preclude interpreting differences between reports as direct contradictions. Our additional evidence concerns the consistency and sensitivity of the associations, rather than discovery of a new susceptibility locus.

The other associated genes provide useful context for this central comparison. *CTLA4*, *HLA-DQA2* and *TUBB* accompanied *TSHR* among genes meeting the full-screen threshold in both GD datasets. The multi-instrument European *CTLA4* estimates relied heavily on rs13030124; with that caveat, *CTLA4*'s direction-consistent associations across all five outcomes support

its use as an autoimmune comparator. *HLA-DQA2* was consistent across GD and ophthalmopathy definitions but not UKB hyperthyroidism, while *TUBB* and *C4A* showed substantial instrument heterogeneity. The strongest BBJ association, *HLA-A*, did not persist in FinnGen GD, illustrating why a smaller discovery *P* value does not necessarily identify a more transferable gene-specific association. Shared instruments and complex MHC linkage further prevent counting every associated expression trait as an independent causal target.

The lower-odds direction for *TSHR* need not contradict the pathogenic role of receptor-stimulating antibodies. Blood transcript abundance, tissue-specific expression and receptor activation are different exposures, and genetic regulatory effects can differ across tissues [25]. Our audit linked the blood expression-increasing rs179252-G allele to rs179247-G in both reference populations. The latter is the GD-protective allele associated with greater thymic transcription in published studies [27, 28] and also increased blood expression in eQTLGen. This directional consistency is compatible with a central-tolerance explanation, but the thymic step was not tested here, and this consistency does not demonstrate that thymic expression mediates the disease association. These linked variants are not independent replications or proven causal regulators. The marked departure of the MR ORs from 1 reflects scaling: rs179252 shifts reconstructed blood expression by only about 0.11 SD, and its per-allele disease ORs were 0.80 (BBJ), 0.77 (UKB), 0.78 (FinnGen ophthalmopathy), 0.77 (FinnGen GD) and 0.82 (strict ophthalmopathy). Dividing these moderate per-allele effects by the small expression effect yields the per-SD ORs, which should not be translated into the effect of experimentally increasing *TSHR* expression. The adipose instruments tag the same intron-1 region as rs179252 and are themselves blood eQTLs, so their concordant estimates re-express this regional association. They cannot address the distinct-variant pattern in UKB and FinnGen GD, establish an adipose-mediated mechanism or provide independent causal replication.

Experimental and clinical studies provide context for relating TSHR-directed immunity to orbital disease. TSHR can undergo cleavage and release its extracellular A-subunit. Expression of this subunit in an adenoviral immunization model induced greater thyroid-stimulating autoimmunity than a minimally cleaving receptor [29]; plasmid immunization subsequently reproduced orbital adipogenesis, fibrosis and muscle pathology in mice [30]. Daser and colleagues further reported enhanced inflammatory and hyaluronan responses to TNFα in orbital fibroblasts from TSHR-immunized mice [31]. Graves ophthalmopathy has also been documented in primarily euthyroid and hypothyroid patients [32], supporting a distinction between thyroid functional status and orbital immune activity. These observations make the ophthalmopathy associations biologically relevant, but they do not show that an A-subunit shed by human orbital fibroblasts, acting alone, causes TED independently of GD. Our instruments do not measure receptor cleavage or antibody activity. The ophthalmopathy comparisons use population controls and overlapping FinnGen endpoints, so neither their MR estimates nor their higher colocalization probabilities establish a TED-specific effect within GD or a difference between GD and TED.

For *IGF1R*, the defensible conclusion is that blood expression-related genetic evidence was limited in this analysis. Concordant ORs above 1 are worth reporting, but the two nominal associations and the imprecise ophthalmopathy estimates do not establish a confirmed susceptibility effect. Loss of nominal significance after omitting single variants further limits confidence in the original estimates, although instrument removal also reduces precision. With 80% power at $P < 0.05$, the FinnGen GD analysis could detect an *IGF1R* OR of about 1.36, so it constrains larger blood-expression effects but not modest ones. The fixed-panel extension found no nominal association for thyroid or cultured-fibroblast *IGF1R*, and directions were inconsistent across datasets. The confidence intervals, single-instrument design and lack of orbital eQTLs preclude excluding a clinically relevant effect. Cultured

fibroblasts are not orbital fibroblasts, and the unestimable tissue combinations are not negative findings. Weaker genetic evidence for *IGF1R* does not argue against IGF-1R blockade: teprotumumab efficacy rests on a randomized trial in active TED [3], and experimental TSHR–IGF-1R crosstalk [5] concerns receptor signaling rather than inherited susceptibility. The results do not demonstrate that *IGF1R* acts only downstream, or that an association would necessarily become detectable using orbital tissue. The small descriptive orbital series cannot resolve either question.

Colocalization does not replace the MR associations, but it constrains causal wording. Strong *TSHR* associations in UKB and FinnGen GD coexisted with H3-dominant regional patterns, and BBJ and strict ophthalmopathy support weakened under a lower shared-variant prior. Thus, association strength and evidence for a shared causal variant were not interchangeable. UKB and FinnGen GD had the most cases of the five datasets; their distinct-variant patterns could reflect finer resolution of a complex regional architecture rather than weaker association, but this was not tested. Ji and colleagues chose a shared-variant prior intended to allow weaker signals to colocalize and accepted PP.H4 above 0.50 [9], whereas our primary settings used $p12 = 10^{-5}$ and PP.H4 ≥ 0.80. These differences further discourage comparing binary colocalization labels across studies without considering the model settings. Single-causal-variant assumptions, ancestry differences and signal coverage can affect these posterior probabilities, particularly in complex immune regions. The regional checks excluded a simple missing-peak explanation for *TSHR* but did not establish the correct causal architecture. For *HLA-DQA2* and *TUBB*, strong MR findings without shared-variant support warrant reporting as associated signals rather than validated expression targets. Multi-signal analysis could clarify some discordance [33], but low PP.H4 neither erases a disease association nor by itself proves that the modeled expression has no causal role.

The sensitivity analyses identify which qualifications matter for the central comparison. Frequency substitution preserved the original BBJ discovery set, whereas *C4B* and *HLA-G* changed significance classifications through altered effect scaling and instrument weighting. Instrument omission identified concentrated evidence for *CTLA4* and reduced statistical support for *IGF1R*. These checks reuse the same data and do not supply independent replication. For researchers evaluating TSHR and IGF-1R, the added answer is that blood expression-related associations are more consistently supported for *TSHR*, while its tissue mechanism and the extent of an inherited *IGF1R* contribution remain unresolved. Independent expression instruments and tissue-relevant regulatory data would help address these uncertainties. Comparing GD patients with and without TED, while accounting for follow-up and selection into the GD group, would address orbital susceptibility directly. The present results inform interpretation and follow-up research, but do not guide treatment selection.

Limitations

The principal limitations involve the exposure and phenotype definitions. Predominantly European blood eQTL effects were applied to BBJ, requiring transferability of expression associations across ancestries. UKB self-reported hyperthyroidism is broader than GD, and an exclusively European source sample was not established. FinnGen's overlapping, population-controlled endpoints do not directly compare GD patients with and without TED; cohort differences and phenotype differences cannot be separated cleanly. Participant overlap with eQTLGen could not be quantified. Cohort-frequency replacement, which uses a release excluding Framingham participants, addresses only part of expression reconstruction. It does not establish correct tissue specificity, remove LD-reference limitations or independently validate rare-variant effects. The orbital series has a single control, no external replication and no inferential role. The GTEx extension had four estimable combinations, each with one

instrument, and predominantly European rather than ancestry-homogeneous exposure data. It reused the disease datasets and lacked tissue-specific regional colocalization.

The analytical limitations include post hoc selection, few instruments and a single-causal-variant colocalization model. *TSHR* has one retained European-reference instrument, precluding multi-instrument pleiotropy tests; several other genes have few variants or heterogeneous estimates. MHC analyses are particularly vulnerable to complex LD and correlated expression, and low PP.H4 can reflect model mismatch or limited signal coverage. Multi-signal colocalization, directionality testing and formal between-phenotype effect tests were not performed. Only 24 selected genes underwent colocalization; therefore its negative classifications cannot be generalized to the entire screen. Discovery and focused follow-up thresholds address specified families of tests, while the expanded scans and sensitivity analyses remain exploratory. Neither a non-significant MR result nor a posterior below 0.80 establishes absence of an effect.

Conclusions

Genetically proxied blood *TSHR* expression showed strong, directionally consistent associations with lower odds of GD and related phenotypes, whereas *IGF1R* evidence was weaker and imprecise. Colocalization supported a shared variant for *TSHR* in BBJ and the ophthalmopathy endpoints, partly prior-dependently, but not in UKB or FinnGen GD, so these data establish association rather than expression-mediated causality. Single-variant adipose instruments from the same locus added concordant estimates but not independent tissue evidence. Weaker genetic evidence for *IGF1R* does not argue against the therapeutic relevance of IGF-1R blockade. Experimental TSHR autoimmunity provides biological context for orbital involvement, but this study does not establish that pathway or isolate TED susceptibility within GD.

Declarations

Funding. This research received no specific grant from any funding agency in the public, commercial, or not-for-profit sectors.

Conflict of interest. The authors declare that they have no conflict of interest.

Ethics approval. This study used publicly available, de-identified summary statistics and an institutionally approved orbital tissue dataset. The in-house orbital transcriptomic component was approved by the Institutional Review Board of Pusan National University Hospital (approval number 2104-018-102) and was conducted in accordance with the Declaration of Helsinki.

Informed consent. Written informed consent was obtained from all individual participants included in the in-house orbital tissue study.

Data availability. Instruments, primary MR and colocalization results, and sensitivity results are supplied as Supplementary Dataset. Analysis code is available from the corresponding author on reasonable request. Public summary statistics analyzed in this study are available from their original repositories: blood *cis*-eQTL data from the eQTLGen Consortium, tissue eQTL data from GTEx v8, the Biobank Japan Graves disease and UK Biobank hyperthyroidism genome-wide association statistics through the GWAS Catalog, and the FinnGen Release 12 GD and ophthalmopathy statistics from FinnGen. The in-house orbital RNA-seq data are available from the corresponding author on reasonable request, subject to institutional and ethical restrictions.

Author contributions. J.P. conceived and designed the study, performed the analyses, and drafted the manuscript. M.-S.K. contributed to data collection and interpretation. K.-H.S. and S.-W.Y. supervised the study and revised the manuscript.

Acknowledgements. We thank the eQTLGen Consortium, the GTEx Consortium, Biobank Japan, the UK Biobank, and the FinnGen study and its participants for making their summary statistics publicly available. We acknowledge the GWAS Catalog for hosting and distributing the genome-wide association summary statistics used as outcome data.

References

1. Bahn RS. Graves' ophthalmopathy. N Engl J Med. 2010;362:726-738. doi:10.1056/nejmra0905750.
2. Smith TJ, Hegedüs L. Graves' Disease. N Engl J Med. 2016;375:1552-1565. doi:10.1056/nejmra1510030.
3. Douglas RS, Kahaly GJ, Patel A, Sile S, Thompson EHZ, Perdok R, Fleming JC, Fowler BT, Marcocci C, Marinò M, et al. Teprotumumab for the Treatment of Active Thyroid Eye Disease. N Engl J Med. 2020;382:341-352. doi:10.1056/nejmoa1910434.
4. Smith TJ. The insulin-like growth factor-I receptor and its role in thyroid-associated ophthalmopathy. Eye (Lond). 2019;33:200-205. doi:10.1038/s41433-018-0265-2.
5. Krieger CC, Boutin A, Jang D, Morgan SJ, Banga JP, Kahaly GJ, Klubo-Gwiezdzinska J, Neumann S, Gershengorn MC. Arrestin-β-1 Physically Scaffolds TSH and IGF1 Receptors to Enable Crosstalk. Endocrinology. 2019;160:1468-1479. doi:10.1210/en.2019-00055.
6. Smith TJ. Potential Roles of CD34+ Fibrocytes Masquerading as Orbital Fibroblasts in Thyroid-Associated Ophthalmopathy. J Clin Endocrinol Metab. 2019;104:581-594. doi:10.1210/jc.2018-01493.
7. Sanderson E, Glymour MM, Holmes MV, Kang H, Morrison J, Munafò MR, Palmer T, Schooling CM, Wallace C, Zhao Q, et al. Mendelian randomization. Nat Rev Methods Primers. 2022;2:6. doi:10.1038/s43586-021-00092-5.
8. Giambartolomei C, Vukcevic D, Schadt EE, Franke L, Hingorani AD, Wallace C, Plagnol V. Bayesian test for colocalisation between pairs of genetic association studies using summary statistics. PLoS Genet. 2014;10:e1004383. doi:10.1371/journal.pgen.1004383.

9. Ji Q, Xu H, Chen H, Chen X, Wang S, Zou J. Pathogenic genes associated with immune-related genes in graves' disease: a multi-omics Mendelian randomization analysis. Sci Rep. 2025;15:37875. doi:10.1038/s41598-025-21754-4.
10. Li Y, Chen L, Lin S, An W, Miao L, Wan M, Zhang B. Inflammatory Proteins Mediate the Effect of Gut Microbiota on Graves' Ophthalmopathy: A Mendelian Randomization Study. Transl Vis Sci Technol. 2025;14:34. doi:10.1167/tvst.14.6.34.
11. Zhang H, Jiang S, Zhu T, Liu Y, Li J, Fang S, Li Y, Sun J, He X, Tong C, et al. Thyroid-stimulating hormone receptor mediates peripheral-central neuroimmune crosstalk in autoimmune thyroid diseases. BMC Med. 2026;24:404. doi:10.1186/s12916-026-04957-y.
12. Skrivankova VW, Richmond RC, Woolf BAR, Yarmolinsky J, Davies NM, Swanson SA, VanderWeele TJ, Higgins JPT, Timpson NJ, Dimou N, et al. Strengthening the Reporting of Observational Studies in Epidemiology Using Mendelian Randomization: The STROBE-MR Statement. JAMA. 2021;326:1614-1621. doi:10.1001/jama.2021.18236.
13. Võsa U, Claringbould A, Westra HJ, Bonder MJ, Deelen P, Zeng B, Kirsten H, Saha A, Kreuzhuber R, Yazar S, et al. Large-scale cis- and trans-eQTL analyses identify thousands of genetic loci and polygenic scores that regulate blood gene expression. Nat Genet. 2021;53:1300-1310. doi:10.1038/s41588-021-00913-z.
14. Sakaue S, Kanai M, Tanigawa Y, Karjalainen J, Kurki M, Koshiba S, Narita A, Konuma T, Yamamoto K, Akiyama M, et al. A cross-population atlas of genetic associations for 220 human phenotypes. Nat Genet. 2021;53:1415-1424. doi:10.1038/s41588-021-00931-x.

15. Dönertaş HM, Fabian DK, Valenzuela MF, Partridge L, Thornton JM. Common genetic associations between age-related diseases. Nat Aging. 2021;1:400-412. doi:10.1038/s43587-021-00051-5.
16. Kurki MI, Karjalainen J, Palta P, Sipilä TP, Kristiansson K, Donner KM, Reeve MP, Laivuori H, Aavikko M, Kaunisto MA, et al. FinnGen provides genetic insights from a well-phenotyped isolated population. Nature. 2023;613:508-518. doi:10.1038/s41586-022-05473-8.
17. Finan C, Gaulton A, Kruger FA, Lumbers RT, Shah T, Engmann J, Galver L, Kelley R, Karlsson A, Santos R, et al. The druggable genome and support for target identification and validation in drug development. Sci Transl Med. 2017;9:eaag1166. doi:10.1126/scitranslmed.aag1166.
18. Chang CC, Chow CC, Tellier LC, Vattikuti S, Purcell SM, Lee JJ. Second-generation PLINK: rising to the challenge of larger and richer datasets. Gigascience. 2015;4:7. doi:10.1186/s13742-015-0047-8.
19. 1000 Genomes Project Consortium, Auton A, Brooks LD, Durbin RM, Garrison EP, Kang HM, Korbel JO, Marchini JL, McCarthy S, McVean GA, et al. A global reference for human genetic variation. Nature. 2015;526:68-74. doi:10.1038/nature15393.
20. Zhu Z, Zhang F, Hu H, Bakshi A, Robinson MR, Powell JE, Montgomery GW, Goddard ME, Wray NR, Visscher PM, et al. Integration of summary data from GWAS and eQTL studies predicts complex trait gene targets. Nat Genet. 2016;48:481-487. doi:10.1038/ng.3538.
21. Hemani G, Zheng J, Elsworth B, Wade KH, Haberland V, Baird D, Laurin C, Burgess S, Bowden J, Langdon R, et al. The MR-Base platform supports systematic causal inference across the human phenome. Elife. 2018;7:e34408. doi:10.7554/elife.34408.

22. Bowden J, Davey Smith G, Burgess S. Mendelian randomization with invalid instruments: effect estimation and bias detection through Egger regression. Int J Epidemiol. 2015;44:512-525. doi:10.1093/ije/dyv080.
23. Bowden J, Davey Smith G, Haycock PC, Burgess S. Consistent Estimation in Mendelian Randomization with Some Invalid Instruments Using a Weighted Median Estimator. Genet Epidemiol. 2016;40:304-314. doi:10.1002/gepi.21965.
24. Wallace C. Eliciting priors and relaxing the single causal variant assumption in colocalisation analyses. PLoS Genet. 2020;16:e1008720. doi:10.1371/journal.pgen.1008720.
25. GTEx Consortium. The GTEx Consortium atlas of genetic regulatory effects across human tissues. Science. 2020;369:1318-1330. doi:10.1126/science.aaz1776.
26. Ueda H, Howson JM, Esposito L, Heward J, Snook H, Chamberlain G, Rainbow DB, Hunter KM, Smith AN, Di Genova G, et al. Association of the T-cell regulatory gene CTLA4 with susceptibility to autoimmune disease. Nature. 2003;423:506-511. doi:10.1038/nature01621.
27. Colobran R, Armengol Mdel P, Faner R, Gärtner M, Tykocinski LO, Lucas A, Ruiz M, Juan M, Kyewski B, Pujol-Borrell R. Association of an SNP with intrathymic transcription of TSHR and Graves' disease: a role for defective thymic tolerance. Hum Mol Genet. 2011;20:3415-3423. doi:10.1093/hmg/ddr247.
28. Marín-Sánchez A, Álvarez-Sierra D, González O, Lucas-Martin A, Sellés-Sánchez A, Rudilla F, Enrich E, Colobran R, Pujol-Borrell R. Regulation of TSHR Expression in the Thyroid and Thymus May Contribute to TSHR Tolerance Failure in Graves' Disease Patients via Two Distinct Mechanisms. Front Immunol. 2019;10:1695. doi:10.3389/fimmu.2019.01695.

29. Chen CR, Pichurin P, Nagayama Y, Latrofa F, Rapoport B, McLachlan SM. The thyrotropin receptor autoantigen in Graves disease is the culprit as well as the victim. J Clin Invest. 2003;111:1897-1904. doi:10.1172/JCI17069.
30. Berchner-Pfannschmidt U, Moshkelgosha S, Diaz-Cano S, Edelmann B, Görtz GE, Horstmann M, Noble A, Hansen W, Eckstein A, Banga JP. Comparative Assessment of Female Mouse Model of Graves' Orbitopathy Under Different Environments, Accompanied by Proinflammatory Cytokine and T-Cell Responses to Thyrotropin Hormone Receptor Antigen. Endocrinology. 2016;157:1673-1682. doi:10.1210/en.2015-1829.
31. Daser A, Gulbins A, Horstmann M, Kalisch J, Nolte I, Hashemi Arani F, Lang S, Mattheis S, Bechrakis NE, Banga JP, et al. TNFα drives disease-specific chemokine networks and orbital tissue remodeling in a preclinical model of thyroid eye disease. Eur Thyroid J. 2026;15:ETJ250407. doi:10.1530/ETJ-25-0407.
32. Eckstein AK, Lösch C, Glowacka D, Schott M, Mann K, Esser J, Morgenthaler NG. Euthyroid and primarily hypothyroid patients develop milder and significantly more asymmetrical Graves ophthalmopathy. Br J Ophthalmol. 2009;93:1052-1056. doi:10.1136/bjo.2007.137265.
33. Wallace C. A more accurate method for colocalisation analysis allowing for multiple causal variants. PLoS Genet. 2021;17:e1009440. doi:10.1371/journal.pgen.1009440.

Figure legends

Figure 1. Study design and BBJ discovery. (A) Original blood-expression screen and subsequent disease-definition comparisons. FinnGen endpoints overlap and are not independent replications. (B) One point per estimable gene (n = 2,234), positioned by the GRCh37 coordinate of its strongest selected expression instrument. The horizontal line is P = 0.05/2,544, equivalent to $-\log10(P) \approx 4.707$. Thirteen discoveries and *IGF1R* are labelled. This is a gene-level MR plot, not a SNP GWAS Manhattan plot. The threshold adjusts the original discovery screen, not every later exploratory analysis.

Figure 2. *TSHR* and *IGF1R* across disease definitions, with *CTLA4* as comparator. (A) Wald/IVW odds ratios and 95% CIs for all 15 gene–dataset comparisons. The horizontal axis is logarithmic and the reference is OR = 1. Bold P values and an asterisk indicate nominal P < 0.05, not Bonferroni significance. ORs describe reconstructed blood expression, not a treatment intervention. (B) Aligned colocalization probabilities under p1 = p2 = 10^{-4} and p12 = 10^{-5}. H0/H1 denotes no association or disease association only; H2 expression only; H3 distinct variants; H4 a shared variant. Bold H4 numbers indicate ≥0.80. MR P values and posterior probabilities answer different questions.

Figure 3. Regional evidence and *TSHR* prior sensitivity. (A) Primary PP.H4 for 24 selected genes and five datasets. Borders mark PP.H4 ≥ 0.80, not P-value significance. All lettering is black, and probabilities <0.001 retain their full precision in the workbook. Row dividers separate the three focal/comparator genes, the remaining BBJ discoveries, nine additional FinnGen GD discoveries, and sensitivity-selected *HLA-G*. Only these selected genes underwent colocalization. The low BBJ *IGLV* probabilities are limited by absence of the lead expression variant. (B) *TSHR* PP.H4 at p12 = 10^{-5}, 5×10^{-6} and 10^{-6}; p1 = p2 =

641 10^{-4}. The dashed line marks the reporting threshold of 0.80. FinnGen GO and GO strict are

642 overlapping ophthalmopathy endpoints; GD is E4_GRAVES_STRICT.

Table 1. Data sources

Dataset / endpoint	Cases	Controls	Exposure or clinical role
eQTLGen blood expression	—	—	Exposure: up to 31,684; predominantly European
BBJ GD (GCST90018627)	2,809	172,656	Discovery; East Asian
FinnGen GD (E4_GRAVES_STRICT)	3,962	496,386	Post hoc GD comparison; Finnish
UKB hyperthyroidism (GCST90038636)	3,731	480,867	Self-report; ancestry not established as exclusively European
FinnGen GO (GRAVES_OPHT)	858	499,490	Original ophthalmopathy comparison; Finnish
FinnGen GO strict (E4_GRAVES_OPHT_STRICT)	753	499,595	Post hoc definition sensitivity; Finnish

GD, Graves disease; GO, Graves ophthalmopathy; HT, hyperthyroidism/thyrotoxicosis. The three FinnGen R12 endpoints overlap and use population non-cases for their respective definitions. Strict is the source endpoint label, not proof of perfect specificity. The descriptive orbital series (four TED patients, one control) is separate from the genetic datasets.

Table 2. *TSHR*, *IGF1R* and *CTLA4*

Gene	Dataset	IVs	OR (95% CI)	*P* value	PP.H2	PP.H3	PP.H4
TSHR	BBJ GD	1	0.12 (0.07–0.21)	**1.09×10^{-14}**	<0.001	0.049	**0.951**
TSHR	FinnGen GD	1	0.08 (0.05–0.12)	**3.19×10^{-32}**	<0.001	0.736	0.264
TSHR	UKB HT	1	0.09 (0.06–0.14)	**8.77×10^{-28}**	<0.001	0.774	0.226
TSHR	FinnGen GO	1	0.10 (0.04–0.24)	**2.82×10^{-7}**	<0.001	0.014	**0.986**
TSHR	FinnGen GO strict	1	0.15 (0.06–0.39)	**9.02×10^{-5}**	0.027	0.082	**0.891**
IGF1R	BBJ GD	4	1.56 (1.07–2.28)	**0.0212**	0.690	0.236	0.073
IGF1R	FinnGen GD	3	1.07 (0.86–1.33)	0.539	0.731	0.259	0.010
IGF1R	UKB HT	4	1.35 (1.07–1.70)	**0.0117**	0.400	0.196	0.404
IGF1R	FinnGen GO	3	1.41 (0.85–2.33)	0.182	0.623	0.346	0.032
IGF1R	FinnGen GO strict	3	1.52 (0.92–2.49)	0.0996	0.571	0.391	0.038
CTLA4	BBJ GD	1	0.18 (0.11–0.27)	**5.45×10^{-15}**	<0.001	0.799	0.201
CTLA4	FinnGen GD	2	0.17 (0.08–0.36)	**2.15×10^{-6}**	<0.001	0.746	0.254
CTLA4	UKB HT	2	0.21 (0.08–0.57)	**0.0022**	<0.001	0.047	**0.953**
CTLA4	FinnGen GO	2	0.17 (0.04–0.68)	**0.0118**	<0.001	0.022	**0.978**
CTLA4	FinnGen GO strict	2	0.17 (0.05–0.64)	**0.00888**	<0.001	0.021	**0.979**

GD, Graves disease; GO, Graves ophthalmopathy; HT, hyperthyroidism/thyrotoxicosis. IVs, instrumental variables (retained genetic variants). Wald ratio for one IV; random-effects IVW for multiple IVs. ORs are per approximately one SD higher reconstructed blood expression. Bold *P* values denote nominal $P < 0.05$ only. Discovery uses 0.05/2,544. PP.H2 supports an expression-only signal; PP.H3 supports distinct variants; PP.H4 supports one shared variant. Bold PP.H4 denotes ≥0.80 under $p12 = 10^{-5}$. These are probabilities, not *P* values. *CTLA4* multi-IV results are concentrated in rs13030124; *TSHR* is single-IV in all datasets.

Table 3. Screen and supporting analyses

A. BBJ discoveries and FinnGen GD comparison

BBJ discovery gene	BBJ OR (95% CI)	BBJ *P*	FinnGen GD OR (95% CI)	FinnGen GD *P*	Focused support	GD PP.H4
HLA-A	1.98 (1.73–2.27)	2.60×10^{-23}	1.08 (0.77–1.53)	0.648	No	<0.001
HLA-DQA2	2.39 (1.95–2.93)	8.30×10^{-17}	1.90 (1.64–2.20)	1.41×10^{-17}	Yes	<0.001
CTLA4	0.18 (0.11–0.27)	5.45×10^{-15}	0.17 (0.08–0.36)	2.15×10^{-6}	Yes	0.254
TSHR	0.12 (0.07–0.21)	1.09×10^{-14}	0.08 (0.05–0.12)	3.19×10^{-32}	Yes	0.264
C4A	0.45 (0.35–0.57)	2.23×10^{-10}	0.41 (0.26–0.63)	4.97×10^{-5}	Yes	<0.001
HSD3B7	0.29 (0.18–0.47)	2.04×10^{-7}	0.90 (0.72–1.13)	0.372	No	0.011
TUBB	1.49 (1.28–1.74)	3.14×10^{-7}	3.28 (2.09–5.17)	2.68×10^{-7}	Yes	<0.001
VKORC1	0.13 (0.06–0.29)	6.35×10^{-7}	NA	NA	NA	0.011
TNFSF14	0.63 (0.52–0.76)	1.48×10^{-6}	0.77 (0.65–0.92)	0.00361	Yes	0.303
PRSS36	6.00 (2.87–12.57)	2.02×10^{-6}	1.40 (1.00–1.96)	0.0490	No	0.063
MAPKAPK5	0.977 (0.967–0.987)	5.28×10^{-6}	0.990 (0.929–1.055)	0.751	No	0.041
PSMB8	1.25 (1.13–1.38)	6.86×10^{-6}	0.96 (0.85–1.08)	0.512	No	<0.001
IFNGR1	2.10 (1.51–2.91)	9.41×10^{-6}	1.62 (1.18–2.24)	0.00330	Yes	0.313

B. Additional FinnGen GD discoveries

Additional gene	GD IVs	GD OR (95% CI)	GD *P*	BBJ PP.H4	GD PP.H4	GO strict PP.H4
APOM	1	0.05 (0.04–0.06)	1.43×10^{-108}	<0.001	<0.001	0.227
NEU1	1	0.04 (0.03–0.06)	1.43×10^{-108}	<0.001	<0.001	0.061
IGLV2-18	1	0.56 (0.48–0.66)	7.52×10^{-12}	0.029	0.020	0.100
IGLV3-12	1	1.20 (1.14–1.26)	7.52×10^{-12}	0.019	0.022	0.101
TNF	1	0.60 (0.51–0.71)	4.65×10^{-10}	<0.001	<0.001	<0.001
HIST1H4E	1	2.54 (1.88–3.43)	1.51×10^{-9}	<0.001	<0.001	0.007
LTA	1	0.46 (0.35–0.61)	3.75×10^{-8}	<0.001	<0.001	<0.001
LTB	1	0.18 (0.10–0.36)	4.89×10^{-7}	<0.001	<0.001	<0.001
C4B	2	1.63 (1.31–2.03)	1.10×10^{-5}	<0.001	<0.001	0.311

C. Frequency-sensitive results

Gene / GD analysis	Reference OR (95% CI)	Reference *P*	Cohort-frequency OR (95% CI)	Cohort *P*	Interpretation
C4B	1.63 (1.31–2.03)	1.10×10^{-5}	1.75 (1.31–2.33)	1.48×10^{-4}	Heterogeneous; loses screen significance
HLA-G	0.86 (0.72–1.03)	0.107	0.60 (0.47–0.75)	1.72×10^{-5}	Weight-sensitive; single-variant dependence

Panel A: focused support requires concordant direction and $P < 0.05/13$; 7 of 13 qualify. NA is unavailable, not a negative result. Panel B: nine additional reference-frequency FinnGen GD discoveries at $P < 0.05/2{,}544$; post hoc and not independent of the full screen. Seven of the nine are instrumented within the extended MHC region on chromosome 6. *APOM*/*NEU1* share rs3101018; *IGLV2-18*/*IGLV3-12* share rs5759410. The lead *IGLV* expression variant is absent in BBJ, weakening interpretation of its low PP.H4. Panel C: *HLA-G* was added because of frequency sensitivity, not primary discovery. Neither *C4B* nor *HLA-G* gains strong GD colocalization. All 24 genes and five datasets are available in the workbook.

Figure 1. Study design and BBJ discovery

A Blood expression instruments
4,462 druggable genes considered
2,544 genes; 6,135 SNP–gene records

→

BBJ GD discovery
2,234 estimable genes; 13 discoveries
Threshold P < 0.05/2,544

→

Disease-definition comparisons
FinnGen GD, UKB hyperthyroidism
Two overlapping GO definitions

Regional follow-up: TSHR and IGF1R at the centre; CTLA4 comparator; 24 selected genes in total.

B BBJ gene-level MR associations

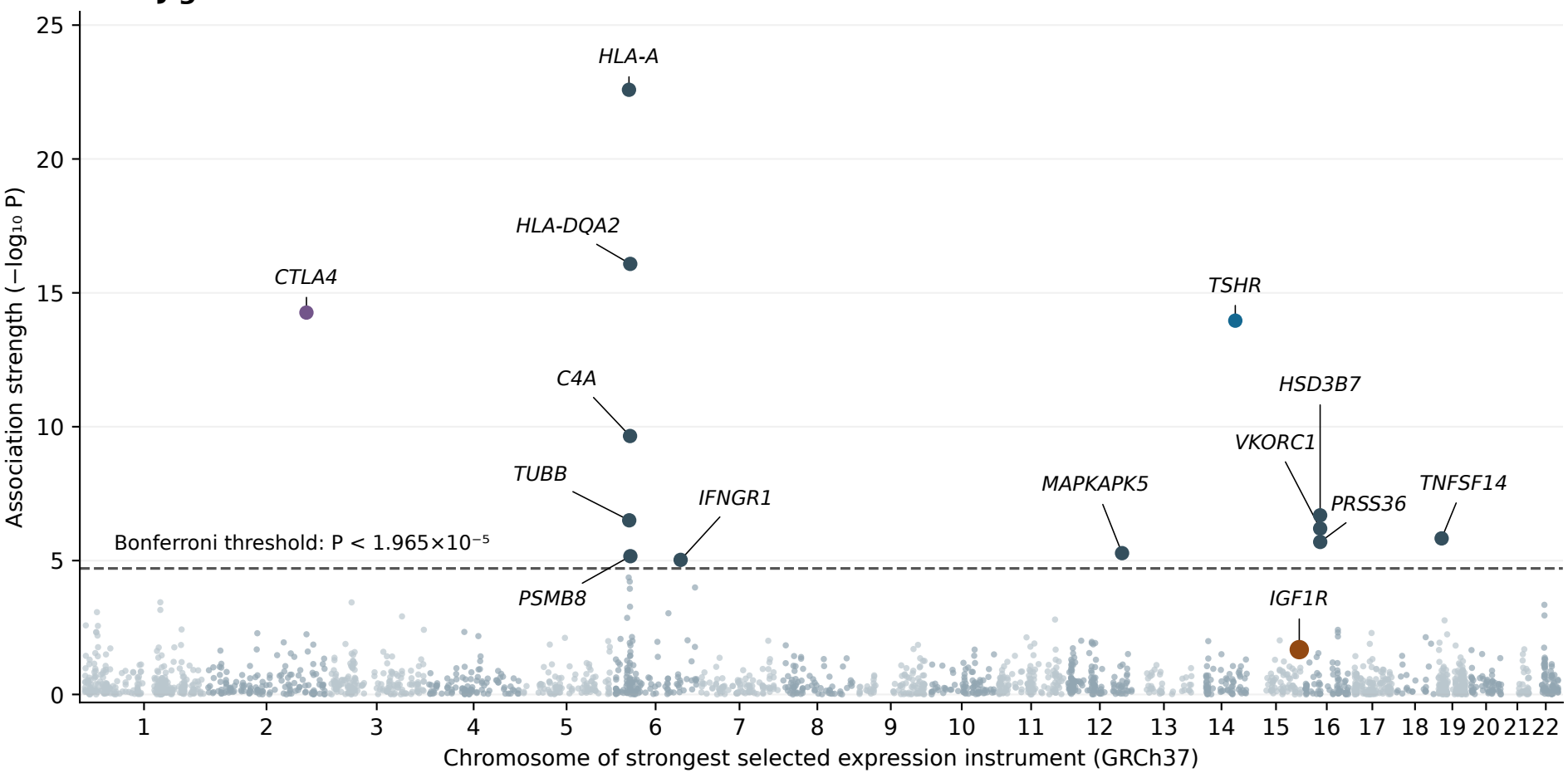


Figure 2. TSHR and IGF1R across disease definitions, with CTLA4 as comparator

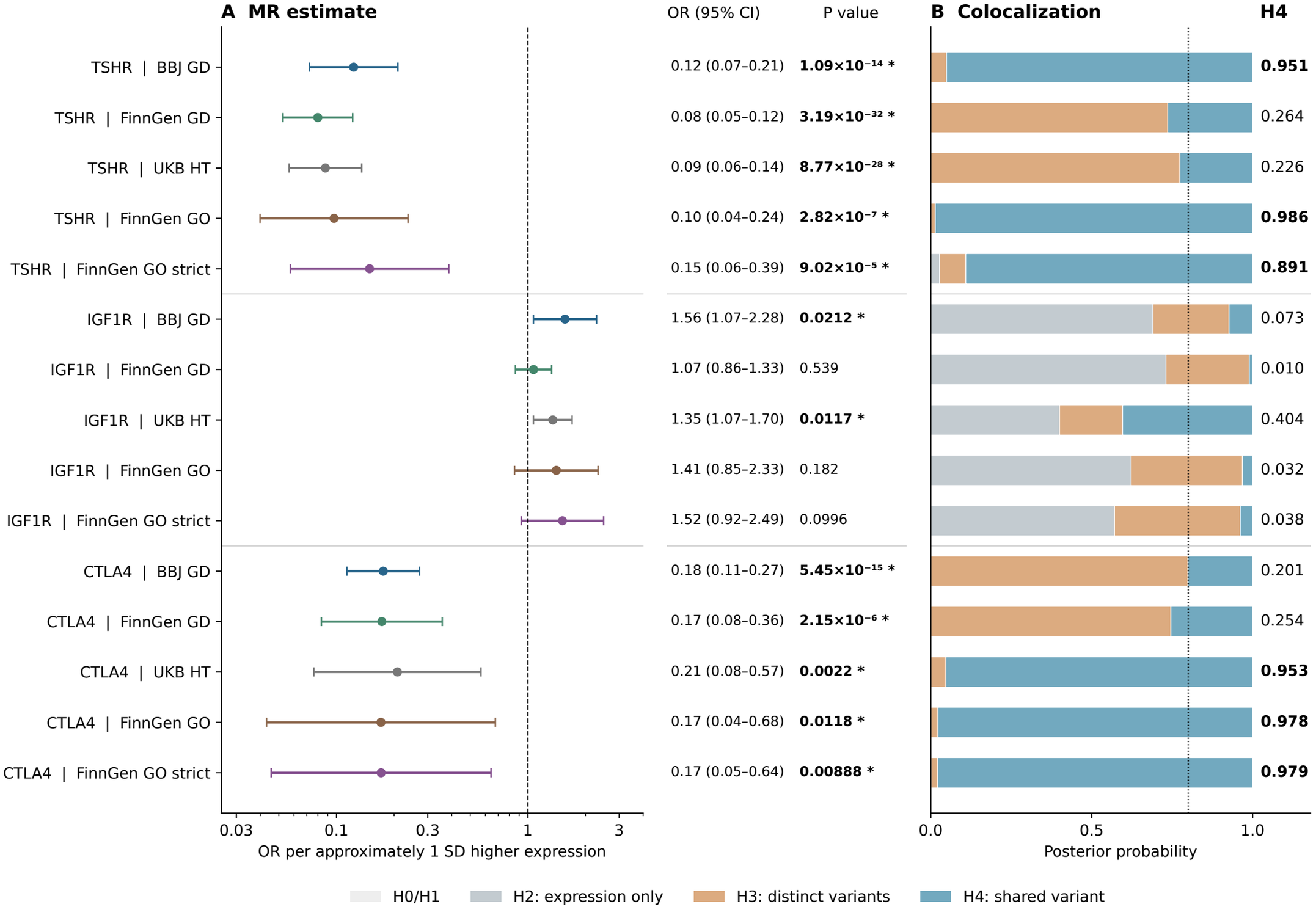
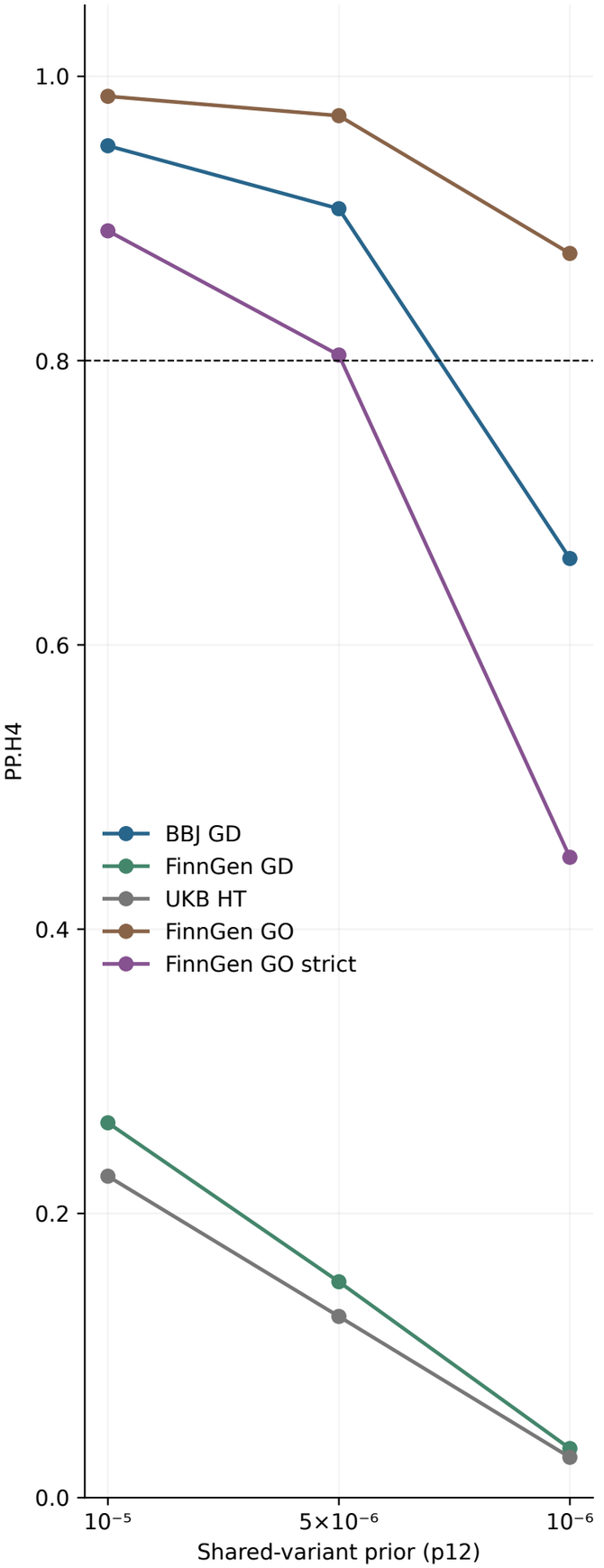


Figure 3. Regional genetic evidence and TSHR prior sensitivity

A Shared-variant probability (PP.H4)

	BBJ GD	FinnGen GD	UKB HT	FinnGen GO	FinnGen GO strict
TSHR	0.951	0.264	0.226	0.986	0.891
IGF1R	0.073	0.010	0.404	0.032	0.038
CTLA4	0.201	0.254	0.953	0.978	0.979
HLA-A	<0.001	<0.001	<0.001	<0.001	<0.001
HLA-DQA2	<0.001	<0.001	<0.001	<0.001	<0.001
C4A	<0.001	<0.001	<0.001	0.030	0.326
HSD3B7	0.636	0.011	0.095	0.026	0.046
TUBB	<0.001	<0.001	<0.001	<0.001	<0.001
VKORC1	0.388	0.011	0.030	0.030	0.048
TNFSF14	0.994	0.303	0.237	0.017	0.018
PRSS36	0.168	0.063	0.115	0.037	0.033
MAPKAPK5	<0.001	0.041	0.005	0.026	0.029
PSMB8	<0.001	<0.001	<0.001	<0.001	<0.001
IFNGR1	0.989	0.313	0.052	0.020	0.021
APOM	<0.001	<0.001	<0.001	0.006	0.227
NEU1	<0.001	<0.001	<0.001	0.003	0.061
IGLV2-18	0.029	0.020	0.024	0.082	0.100
IGLV3-12	0.019	0.022	0.024	0.083	0.101
TNF	<0.001	<0.001	<0.001	<0.001	<0.001
HIST1H4E	<0.001	<0.001	<0.001	0.004	0.007
LTA	<0.001	<0.001	<0.001	<0.001	<0.001
LTB	<0.001	<0.001	<0.001	<0.001	<0.001
C4B	<0.001	<0.001	<0.001	0.030	0.311
HLA-G	<0.001	<0.001	<0.001	<0.001	0.001

B TSHR prior sensitivity



Supplementary material

TSHR and IGF1R in Graves disease and thyroid eye disease: genetic evidence from blood expression

Supplementary methods

Dataset definitions and analysis chronology

BBJ GD (GCST90018627) and UKB self-reported hyperthyroidism/thyrotoxicosis (GCST90038636) retained the source sample sizes and estimates. UKB used BOLT-LMM linear-model statistics; its report included adjustment for sex, age, BMI, assessment centre, ethnicity, genotyping batch and 20 principal components. The available documentation did not establish an exclusively European analysis sample. FinnGen R12 GWAS counts were 858/499,490 for GRAVES_OPHT, 3,962/496,386 for E4_GRAVES_STRICT and 753/499,595 for E4_GRAVES_OPHT_STRICT. Counts refer to the analyzed GWAS files, not later phenotype-browser totals. Strict GD combines specified GD/ophthalmopathy diagnostic criteria with the Graves condition and exclusion of amiodarone exposure; strict ophthalmopathy requires its specified ophthalmopathy and thyrotoxicosis codes. The exact endpoint definitions and source manifests are retained with the analysis verification records. None supplies GD-only controls.

The original three-outcome analysis preceded the two FinnGen extensions. The expanded analysis plan was recorded before the new endpoint associations were computed, but it was not a prospective registration of the complete study. The 24 colocalization genes comprise 13 BBJ discoveries, *IGF1R*, nine additional primary FinnGen GD discoveries, and frequency-sensitive *HLA-G*. No new discovery threshold was optimized from these results.

Instrument selection and effect reconstruction

Significant cis-eQTL instruments ($P < 5\times10^{-8}$ within ±1 Mb of the gene) were clumped at $r^2 < 0.001$ within 10 Mb using the European 1000 Genomes Phase 3 reference (n = 503). Selected instruments and GRCh37 positions are in the Instruments worksheet. $SE = 1/\sqrt{[2f(1-f)(N + Z^2)]}$ and $\beta = Z \times SE$, where f is the assessed-allele frequency and N is the variant-specific expression sample size. Unit expression variance was assumed. Variant $F = Z^2$. The estimate is approximately per expression SD, not per SD of the genetic predictor.

TwoSampleMR harmonise_data(action = 2) aligned alleles and excluded ambiguous palindromic variants with MAF > 0.42. Missing outcome estimates were not imputed. BBJ and FinnGen retained log-odds scales; UKB β and SE were each divided once by $\mu(1-\mu)$, with $\mu = 3{,}731/484{,}598$. GRCh37 expression and GRCh38 FinnGen coordinates were not directly joined: identifiers and allele pairs

were checked, with duplicate and alias audits. Source covariate adjustments were retained; patient-level re-adjustment and subgroup analysis were unavailable.

MR estimators and instrument diagnostics

For a single instrument, the Wald coefficient is β_Y/β_X and its first-order SE is $SE_Y/|\beta_X|$. Multiple-instrument MR used inverse-variance weighted regression through the origin. Multiplicative random-effects SEs included under-dispersion correction, so heterogeneity could widen the interval. Normal two-sided tests and 95% CIs were used for the primary estimators. The alternative-estimator worksheet preserves weighted median, weighted mode and MR-Egger results for the original three outcomes; mode and Egger intervals use the corresponding t distributions. These alternative estimators were not rerun for all new endpoints.

Post hoc leave-one-out analyses applied IVW when at least two variants remained and the Wald ratio when one remained. Single-instrument *TSHR* and BBJ *CTLA4* were ineligible. Original three-outcome results, new-endpoint results for the fixed 14-gene set, and 72 *C4B*/*HLA-G* omission estimates under two frequency sources are retained in separately named worksheets. They reuse the same association data and are not independent replications. *C4B*/*HLA-G* instrument weights use β_X^2/SE_Y^2, normalized within each full model. A single-SNP *P* value can be much smaller than the full IVW *P* value when heterogeneity inflates the full-model SE.

TSHR clumping sensitivity used eQTL thresholds of $5\times10^{-8}/5\times10^{-6}$, r^2 thresholds of 0.001/0.01/0.1, and European (n = 503) or East Asian (n = 504) reference panels. Primary settings selected one European and two East Asian variants, and rs179252 was retained in every setting. Chromosome 16p11.2 analyses found high East Asian LD among rs4889606, rs34649473 and rs78924645; conditional analysis supported regional dependence rather than three independent gene effects.

TSHR allele alignment and reference LD

In a post hoc audit, original full eQTLGen and disease GWAS files were re-extracted for rs179252 and rs179247. Their GRCh37 positions were 14:81435985 and 14:81432546, and GRCh38 positions were 14:80969641 and 14:80966202. Both non-palindromic allele pairs matched across sources. UKB effects were reversed to the G allele after the existing scale conversion; BBJ and FinnGen required no reversal. Exposure coefficients and first-order Wald estimates were independently reconstructed. Original GWAS *P* values and recalculated MR *P* values were retained separately.

G-allele counts at both variants were positively correlated in European (n = 503; r = 0.978322812; r^2 = 0.957115524) and East Asian (n = 504; r = 0.995803919; r^2 = 0.991625445) references, without missing genotypes. Direct BED decoding agreed with PLINK dosage output. PLINK haplotype estimates supported G–G and T–A coupling; these were inferred from unphased genotypes, not

directly observed phased haplotypes. Genotype-based r^2 and haplotype-based estimates were kept distinct. Alignment to the G allele resolved the negative default European correlation obtained when counting T at rs179252. The TSHR_allele_audit worksheet retains both eQTL records, both variants across five disease outcomes, all four inferred haplotypes in each population, and MR reproduction. This audit does not identify a causal variant or establish mediation in thymus or orbit.

Colocalization hypotheses and regional inputs

coloc.abf received beta/varbeta data for disease (dataset 1, case-control type) and expression (dataset 2, quantitative type, sdY = 1). Outcome allele frequency was not used in the Bayes factors. Complete available cis-eQTL statistics, not just significant eQTLs, were matched by identifier and allele compatibility. H0 denotes neither trait associated; H1 disease only; H2 expression only; H3 both traits associated through distinct variants; H4 both traits associated through one shared variant. PP.Hk is the posterior probability of hypothesis k, conditional on the model and priors. Dataset order is essential to interpreting H1 and H2.

Primary priors were $p1 = p2 = 10^{-4}$ and $p12 = 10^{-5}$; sensitivity p12 values were 5×10^{-6} and 10^{-6}. The PP.H4 ≥ 0.80 threshold was a reporting convention, not a significance test. H0–H4 sum to one apart from rounding. Values displayed as <0.001 are nonzero full-precision probabilities in the workbook. The top_snp is conditional on H4, not necessarily the disease lead variant. H3 is a competing distinct-variant model, not a count of causal signals. Multi-signal colocalization was not performed.

Four frequency/availability scenarios were used: original_reference (original available reference frequencies), paired_reference and paired_eqtlgen (the same variants with the respective frequencies), and available_eqtlgen (all matching variants with valid cohort frequencies). There are 1,440 gene–outcome–scenario–prior rows. The additional TSHR analysis comprises four windows × five outcomes × four scenarios × three priors = 240 rows. Windows were the full source cis-region, ±500 kb and ±250 kb around the recorded GRCh37 GenePos anchor, and the five-outcome variant intersection. The full cis-source contained 7,309 eQTL rows; all three FinnGen full-region analyses matched 6,579 variants, and the five-outcome intersection contained 4,030. Raw outcome regional scans checked whether the strongest disease variants survived matching. New candidate coverage checks also examined marginal expression strength; the absent rs5759410 lead in BBJ was checked against variant aliases without finding a replacement.

For *TSHR*, *IGF1R* and *CTLA4*, colocalization and MR in FinnGen GD and strict ophthalmopathy were independently reproduced from re-downloaded FinnGen R12 regional statistics using two separately written implementations, with the original ophthalmopathy endpoint as a control. The allele-compatible variant sets were the same (6,675, 7,850 and 4,993 variants), and the top variant was

the same in all 27 gene–endpoint–prior settings, including the control. The reproduction used gnomAD v2.1.1 European frequencies, which were available for more of these variants than the 1000 Genomes European frequencies used in the reported analysis (6,674, 7,850 and 4,992 analysed, versus 6,579, 7,740 and 4,924). On the control endpoint, PP.H4 changed by at most 4.1×10^{-4}; for *TSHR* this difference came from the additional variants. Reported new-endpoint PP.H4 values were reproduced within 1.2×10^{-3}, and no setting changed side of 0.80. The *TSHR* strict-ophthalmopathy value at p12 = 10^{-6} was 0.449 rather than 0.451 (0.451 on the reported variant set), and *IGF1R* PP.H2 and PP.H3 differed by up to 0.003, as on the control endpoint. MR estimates were reproduced to numerical precision on the original exposure scale.

Frequency substitution and precision benchmarks

The published eQTLGen frequency release covered 26,609 participants, excluding Framingham. Reconstructing β_X and SE_X with fixed Z and N preserves their ratio up to floating-point precision. For a single-IV Wald estimate, frequency scaling changes the effect unit and SE together and preserves the normal-test P value; for IVW, variant weights can change. The new-endpoint MR dataset separates identical-variant scaling from repeat allele harmonization. *C4B* and *HLA-G* were audited with identical retained variants and outcome effects, so their differences arise from scaling and weighting rather than a changed variant set. Large frequency discrepancies are documented, not assumed to be resolved by correct identifier matching.

The detection benchmarks use $(z_{1-\alpha/2} + z_{0.80}) \times SE$ as the approximate 80% minimum detectable absolute log-OR. Alpha was 0.05/2,544 for BBJ and 0.05 for the original UKB/FinnGen comparisons. These are precision summaries using observed SEs, not prospective sample-size calculations, post hoc proof of power, or power of colocalization/the combined filter. The Detection_original worksheet retains these three-outcome results. Applying the same formula to the MR_primary standard errors for FinnGen GD at alpha 0.05/2,544 gave 24.8% and 52.2% of 2,480 estimable genes with 80% power for ORs of 1.5 and 2.0. At alpha 0.05, the detectable *IGF1R* ORs were 1.72 in BBJ, 1.39 in UKB and 1.36 in FinnGen GD.

Fixed-panel tissue extension

The post hoc tissue extension retained a frozen panel of *TSHR* and *IGF1R* in five standard-release GTEx v8 tissues. QTL sample counts were 574 (thyroid), 483 (cultured fibroblasts), 581 (subcutaneous adipose), 469 (visceral adipose) and 670 (whole blood). The original v8 archive supplied significant variant–gene pairs and eGenes records for all tested genes; gene-specific release thresholds exceeded 5×10^{-8} for every eligible combination, permitting complete selection at the fixed threshold. Absence from the tested-gene file was distinguished from a minimum eQTL P value above

the threshold. The standard release includes multiple ancestries; the overall donor cohort was 85.3% European American, not a tissue-specific percentage.

Variants were restricted to ±1 Mb of the source GENCODE v26 TSS, $P < 5\times10^{-8}$, F > 10 and MAF ≥ 0.01. GTEx variant IDs identify GRCh38 REF and ALT, with slope referring to ALT. Actual effect-allele frequencies were reconstructed from each selected eGene MAF and ref_factor, not by treating MAF as ALT frequency. GTEx mappings linked GRCh38 variants to rsIDs and GRCh37 reference coordinates. PLINK clumping used 503 European reference samples, $r^2 < 0.001$ and a 10-Mb window; an independent genotype-based calculation reproduced selection. One of 20 thyroid *IGF1R* candidates was absent from the reference, while its strongest candidate was retained. All selected variants were non-palindromic and matched exactly in the five disease datasets; no proxies or relaxed thresholds were used.

Each of the four estimable gene–tissue combinations retained one SNP. Wald estimates used β_Y/β_X and $SE_Y/|\beta_X|$ with normal two-sided *P* values and 95% CIs; independent calculations reproduced all 20 estimates. The UKB linear-to-log-odds conversion was applied once. ORs represent one unit of inverse-normal-transformed normalized expression; they are not directly comparable to an intervention or to the reconstructed eQTLGen scale. Primary adjusted $P = \min(1, 10P)$ for FinnGen GD; secondary adjusted $P = \min(1, 40P)$ for the other datasets. The 30 unestimable cells remained in the planned families. Single-instrument analyses cannot support heterogeneity, leave-one-out or MR-Egger diagnostics. Significant-only eQTL pairs were not used for colocalization; full regional data were not obtained from the requester-pays resource, and no tissue posterior was calculated. Tissue_extension in the single Supplementary Dataset retains all 50 cells, availability, selected and excluded candidates, harmonized alleles and source statistics.

Orbital expression and reproducibility

Four patients with inactive TED and one control contributed orbital RNA-sequencing data. Orbital adipose tissue (approximately 2 ml, inferolateral compartment) was obtained during orbital decompression for persistent proptosis, diplopia or exposure keratopathy from patients with a documented active phase (Clinical Activity Score ≥3/7) who had completed immunosuppressive treatment, had remained inactive (Clinical Activity Score <3/7) for at least 6 months and had been euthyroid for at least 3 months. The control tissue was the nasal (medial) fat pad of one person without thyroid or autoimmune disease who underwent cosmetic upper blepharoplasty. Samples were snap-frozen; RNA was extracted with the RNeasy Lipid Tissue Mini kit (QIAGEN), and paired-end libraries (200–400 bp inserts) were sequenced on an Illumina NovaSeq platform, with two technical-replicate libraries per donor. Reads were checked with FastQC 0.11.7, trimmed with Trimmomatic 0.38, aligned to GRCh38 with HISAT2 2.1.0 and quantified as TPM with StringTie 2.1.3b. Technical replicates were combined by biological sample. Figure S1 shows sample-level transcript abundance

and log2[(mean TED TPM + 0.01)/(control TPM + 0.01)]. One control cannot estimate control-group biological variance reliably, and no differential-expression *P* value was used. The dataset is descriptive and did not influence candidate selection.

The workbook preserves exported full-precision analytical outputs, not spreadsheet reimplementations of MR. Readme and Column_dictionary identify each sheet, its scope and its columns. Reproduction of a calculation validates implementation, not instrument validity, causal assumptions or clinical generalizability.

Table S1. *TSHR* regional and prior sensitivity

Dataset	Full cis SNPs	Primary PP.H4	Lower-prior PP.H4	PP.H4 range across windows
BBJ GD	4,336	0.951	0.661	0.951–0.951
FinnGen GD	6,579	0.264	0.035	0.264–0.264
UKB HT	5,667	0.226	0.028	0.226–0.226
FinnGen GO	6,579	0.986	0.875	0.986–0.987
FinnGen GO strict	6,579	0.891	0.451	0.891–0.905

Lower prior: p12 = 10^{-6}; p1 = p2 = 10^{-4} throughout. Window range uses the full source cis-region, ±500 kb, ±250 kb and five-outcome intersection at the primary prior. It is not a confidence interval. Full regional disease lead variants were retained. All 240 settings are in TSHR_windows.

Table S2. Frequency-sensitive exploratory candidates

Gene / dataset	Frequency	IVs	OR (95% CI)	*P* value	Q-test *P*
C4B / FinnGen GD	Reference	2	1.63 (1.31–2.03)	1.10×10^{-5}	7.79×10^{-7}
C4B / FinnGen GD	eQTLGen	2	1.75 (1.31–2.33)	1.48×10^{-4}	1.36×10^{-8}
C4B / FinnGen GO strict	Reference	2	1.68 (1.44–1.95)	2.74×10^{-11}	0.122
C4B / FinnGen GO strict	eQTLGen	2	1.80 (1.46–2.23)	6.17×10^{-8}	0.0587
HLA-G / FinnGen GD	Reference	16	0.86 (0.72–1.03)	0.107	9.98×10^{-24}
HLA-G / FinnGen GD	eQTLGen	16	0.60 (0.47–0.75)	1.72×10^{-5}	2.26×10^{-10}
HLA-G / FinnGen GO strict	Reference	16	0.91 (0.76–1.08)	0.267	0.0231
HLA-G / FinnGen GO strict	eQTLGen	16	0.70 (0.53–0.92)	0.0122	0.131

C4B and *HLA-G* frequency sensitivity. Variant sets and outcome effects were held fixed. Frequency changes rescaled expression estimates and IVW weights. Q tests heterogeneity, not directional pleiotropy; its *P* value is not a gene-association *P* value. All 72 omission estimates and instrument weights are provided in the workbook.

Table S3. Tissue-specific expression and FinnGen GD

Gene / tissue	N	IVs	OR (95% CI)	*P* value	Adjusted *P*
TSHR / Thyroid	574	0	No qualifying IV	—	—
IGF1R / Thyroid	574	1	0.76 (0.56–1.02)	0.0692	0.692
TSHR / Cultured fibroblasts	483	0	Not tested	—	—
IGF1R / Cultured fibroblasts	483	1	1.11 (0.89–1.38)	0.359	1.000
TSHR / Subcutaneous adipose	581	1	0.36 (0.31–0.43)	1.28×10^{-34}	1.28×10^{-33}
IGF1R / Subcutaneous adipose	581	0	No qualifying IV	—	—
TSHR / Visceral adipose	469	1	0.50 (0.43–0.58)	4.09×10^{-19}	4.09×10^{-18}
IGF1R / Visceral adipose	469	0	No qualifying IV	—	—
TSHR / Whole blood	670	0	No qualifying IV	—	—
IGF1R / Whole blood	670	0	No qualifying IV	—	—

Fixed-panel tissue extension with FinnGen GD as the primary disease dataset. IVs are retained instrumental variants. ORs are per unit of normalized, inverse-normal-transformed expression. Adjusted $P = \min(1, 10P)$; the ten-comparison denominator includes unestimable cells. Not tested means the gene was absent from the tissue eGenes file; no qualifying IV means the minimum eQTL *P* did not meet 5×10^{-8}. All four estimable combinations used one SNP (F 32.1–61.1), so each *P* value equals that variant's disease-association *P* value. The adipose *TSHR* instruments (rs12101255 and rs74064796, 50 bp apart in *TSHR* intron 1) are themselves blood *TSHR* eQTLs near rs179252 and therefore re-express the regional *TSHR* association rather than providing independent evidence. The 40 secondary comparisons use a separate denominator of 40 and are retained, with all source statistics, in Tissue_extension. Cultured fibroblasts and adipose tissues are not orbital tissue. Original detection benchmarks remain in Detection_original.

Figure S1

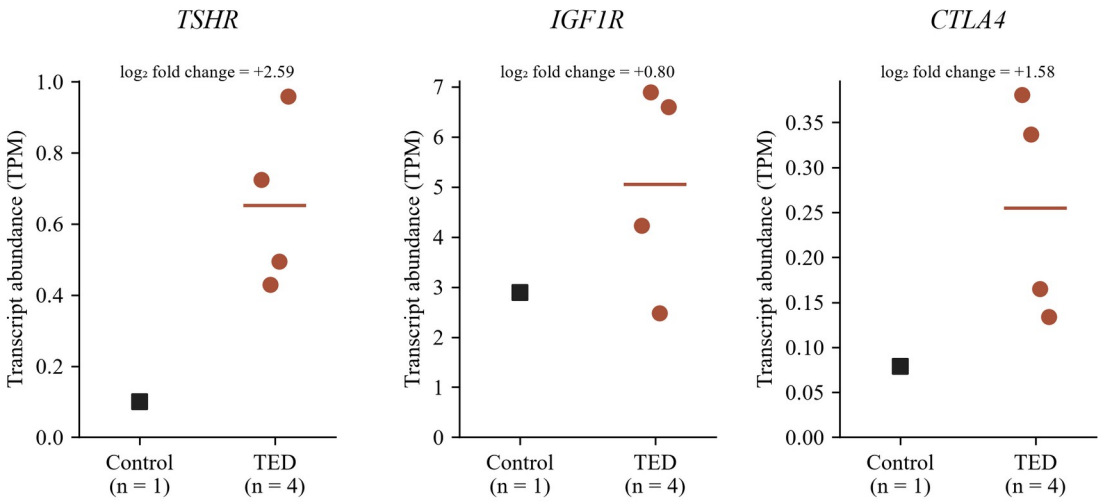


Figure S1. Descriptive orbital expression. Sample-level transcript abundance for *TSHR*, *IGF1R* and *CTLA4* in four TED patients and one control. Points are biological samples; horizontal lines show TED means. Fold changes use log2[(mean TED TPM + 0.01)/(control TPM + 0.01)]. Axes differ by gene. No inferential expression test was performed; these observations do not validate the genetic findings.

Figure S2

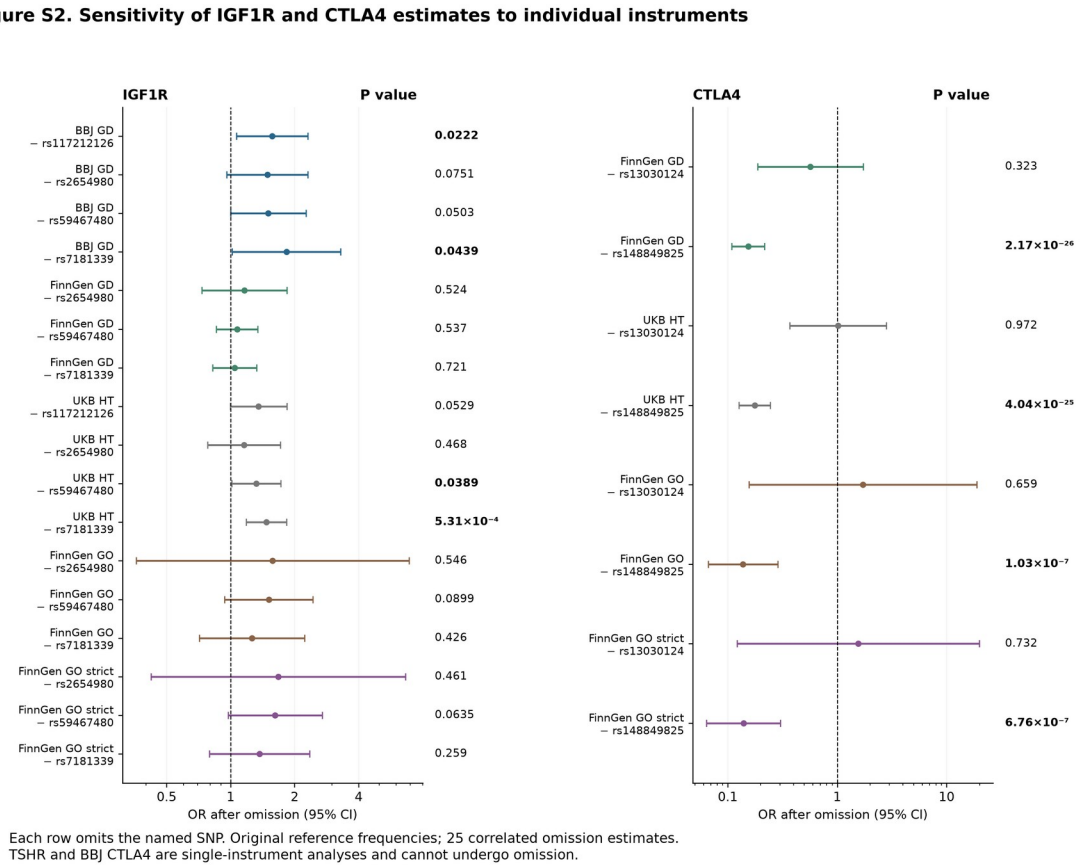


Figure S2. Instrument-omission sensitivity. Twenty-five reference-frequency SNP-omission estimates for *IGF1R* and *CTLA4* across the five datasets. The named SNP is omitted; Wald ratios apply when one remains and IVW when at least two remain. Horizontal lines show 95% CIs; the vertical reference is OR = 1. Bold *P* values mark nominal $P < 0.05$ only. Original full-set estimates are in Table 2. *TSHR* and BBJ *CTLA4* were ineligible because each has one instrument. These correlated post hoc diagnostics are not independent replication.